

HOW TO OPEN A HOMEMAKER & COMPANION SERVICES BUSINESS IN FLORIDA

A Florida-registered Homemaker and Companion Services provider — delivering non-medical companionship, supervision, meal preparation, light housekeeping, laundry, shopping, errands, and transportation so seniors and adults with disabilities can live safely and independently at home.

A step-by-step startup and operations guide

Registration (AHCA) · Screening & training · Getting paid · Operations

Regulatory basis: Section 400.509, Florida Statutes, and Rule 59A-8.025, Florida Administrative Code (Registration Requirements for Homemaker and Companion Services Providers), with the procedures of Chapter 408, Part II, Florida Statutes; registered and overseen by the Florida Agency for Health Care Administration (AHCA). This is a NON-MEDICAL registration — no hands-on personal care. Funding: private pay; Florida Medicaid Statewide Medicaid Managed Care Long-Term Care and the APD iBudget waiver where applicable; Veterans' benefits; and long-term-care insurance.

How to Use This Guide

This guide walks you through opening and operating a Homemaker and Companion Services business in Florida — a provider registered with the Agency for Health Care Administration (AHCA) under Section 400.509, Florida Statutes. A homemaker and companion provider delivers non-medical support: companionship, supervision, and socialization, plus meal preparation, light housekeeping, laundry, shopping, errands, and transportation — so clients can stay safely in their own homes. It is the most accessible, lowest-cost door into Florida's enormous senior-care market: the state registration fee is modest, no nurse or clinical staff is required, and demand is climbing as Florida's senior population grows.

Important — non-medical means non-medical

A homemaker and companion provider is a **non-medical** business. Your staff provide companionship, supervision, homemaking, and transportation — they do NOT provide hands-on personal care. AHCA states plainly that this registration does not allow bathing, dressing, feeding, toileting, transferring, or medication assistance. The moment a client needs hands-on personal care, you refer them to a licensed home health agency or nurse registry — providing that care yourself would require a Home Health Agency license under Chapter 400, Part III. This guide is an educational resource, not legal, tax, or billing advice — verify every step and figure with AHCA and the relevant payer before you act.

The big picture, in one paragraph

Here is the whole business in a nutshell. You register your business with AHCA as a Homemaker and Companion Services provider using AHCA Form 3110-1003 and a modest fee. You Level 2 background screen yourself and every homemaker and companion through the AHCA Clearinghouse before they enter a client's home, and you complete the required HIV/AIDS and (where applicable) dementia and emergency-management training. You find clients through hospitals, senior communities, care managers, elder-law attorneys, and families. And you get paid — mostly private pay to start, with Florida Medicaid Long-Term Care and the APD waiver available as you grow. Register it, screen and train, fill it, bill it — all strictly non-medical.

The path at a glance — step by step

1. Form your business (LLC or corporation) with the Florida Division of Corporations (SunBiz) and get an EIN; register a fictitious name if you'll operate under a different name.
2. Choose a business name that does NOT include restricted words (home health, nursing, medical, personal care, etc.).
3. Decide your geographic service area — the county or counties within your health service planning district.
4. Build your Policy & Procedure Manual, caregiver handbook, and forms (this package) so you're operation-ready and survey-ready.
5. Designate your administrator and prepare for the annual screening affidavit.
6. Complete Level 2 background screening through the AHCA Clearinghouse for owners, the administrator, and staff.
7. Complete required HIV/AIDS training and prepare the HIV/AIDS Training Affidavit (Form 3110-1011).
8. Submit AHCA Form 3110-1003 and the registration fee through AHCA's online licensure system.
9. Respond to any AHCA omissions letter within 21 days; AHCA decides within 60 days of a complete application.
10. Once registered, recruit and train staff, start referral relationships early, and (if billing Medicaid) enroll and set up EVV.

Part 1 — What This Business Is

A Florida homemaker and companion provider delivers non-medical, supportive services that help elderly and disabled adults remain independent at home. Companion services include companionship, supervision,

socialization, accompaniment to appointments, general safety monitoring, and verbal reminders (including medication reminders that prompt but never handle or administer medication). Homemaker services include meal preparation, light housekeeping, laundry, grocery and errand shopping, and transportation. Staff may serve one client or a small group of up to three recipients.

Registration: Homemaker and Companion Services provider, registered with AHCA under Section 400.509, Florida Statutes, and Rule 59A-8.025, F.A.C. It is a registration, not a health-care license — which is exactly why it is the fastest, most affordable way into the home-care market.

With this registration you can employ or contract homemakers and companions and provide non-medical services for pay to elderly and disabled adults in your service area. What you cannot do is provide or advertise hands-on personal care, nursing, therapy, or medication administration. Providing any hands-on personal care — even helping a client into the shower once — crosses into home health agency territory and requires a Home Health Agency license under Chapter 400, Part III. Staying clearly on the non-medical side of that line is both a legal requirement and a simpler, cleaner business to run.

Why this is a strong business to start

Florida has the highest percentage of seniors in the nation — over four million residents are 65 or older, and the state's older population keeps growing. Most people strongly prefer to age in their own homes rather than move to a facility, and families, hospitals, and senior communities are constantly looking for dependable, non-medical help. The homemaker and companion registration lets you meet that demand without hiring nurses, without a Certificate of Need, and with a very low state entry cost. What you are really selling is trust and reliability: screened, trained staff who show up and help people live well at home.

The economics are approachable. Unlike a full home health agency — with its multi-thousand-dollar application fee, surety bond, Director of Nursing, and pre-licensure survey — the homemaker and companion registration is inexpensive to obtain and can even be operated from a home office within Florida. That low barrier is a gift and a risk: it is easy to start, so your edge comes from professionalism — good systems, reliable staff, and a spotless compliance record — which is exactly what this package helps you build.

You can start focused (a particular metro, or a niche such as dementia companionship or post-hospital support) and grow. Many providers begin private-pay and later add Medicaid clients through the Statewide Medicaid Managed Care Long-Term Care program or the APD iBudget waiver. And when you are ready to offer hands-on personal care or skilled services, the natural next step is a Home Health Agency license — a bigger undertaking your established reputation and referral relationships will make far easier to reach.

Who your clients are

It helps to picture who you serve. Some are seniors who live alone and need companionship, meals, and light housekeeping a few hours a day, plus someone to notice if something is wrong. Some are recovering from a hospital stay and need transportation, errands, and supervision for a few weeks. Some have early dementia and need calm companionship and a familiar routine (which triggers dementia training). Some are adults with disabilities who need homemaking and community support. Each comes with a different funding path (Part 6), but all need the same core thing: a dependable, screened companion who treats them with respect — within the non-medical scope.

Choosing your niche

You don't have to serve everyone. The strongest new providers pick a clear focus and build their roster, training, and marketing around it. Common non-medical niches include dementia companionship (calm supervision and routine for clients with early Alzheimer's, which triggers ADRD training), post-hospital and post-surgery support (transportation, errands, meal prep, and supervision to reduce readmissions), companionship for isolated seniors (regular visits, conversation, and outings), and homemaking-focused support (housekeeping, laundry, shopping, and meals). A niche makes you referable: a discharge planner remembers 'the agency that's great with dementia clients' far more easily than a generic one.

Getting a realistic picture of the market

Before you commit, spend a few hours understanding your local market. Who already provides non-medical companion and homemaker services in your county? What do they charge, and what do their reviews say they do well or poorly? Which hospitals, rehab facilities, and senior communities are nearby, and who handles their discharges and placements? Your local Area Agency on Aging (see Statewide Contacts) is a goldmine — it fields constant requests for exactly your services and can tell you where the unmet need is. The goal is not to be intimidated by competitors but to find the gap: the neighborhood, language community, or niche that is underserved and that you can serve better.

The realities and responsibilities

This is a real business with real obligations, and it helps to be clear-eyed about them from day one. You are responsible for screening and training every person who enters a client's home, for supervising service and keeping accurate records, for meeting payroll and labor law, and for staying inside the non-medical scope no matter how much a client wishes you could do more. You will handle vulnerable adults' trust, sometimes their house keys and errand money, and occasionally difficult family dynamics. None of this is a reason not to start — it is a reason to start with good systems. Providers who take compliance and documentation seriously sleep well, pass surveys, and build referral reputations; those who wing it eventually get a complaint survey they are not ready for. This package exists to put those systems in place for you.

Part 2 — Registering with AHCA

Registration is the heart of getting started, and it is refreshingly straightforward compared with a full home health license. You apply on the Health Care Licensing Application, Homemaker and Companion Services Provider (AHCA Form 3110-1003), through AHCA's online licensure system. Provider name, address, and telephone number will be published on Florida Health Finder, so keep them accurate.

The fee and timeline

The registration fee is **\$50.75 for a two-year registration** (verify current — AHCA fees can change). If a renewal is submitted fewer than 60 days before expiration, a late fee (recently \$25.38) applies and must be paid before AHCA issues the registration. There are additional, separate costs for background screening (below). After you submit, AHCA reviews the application and, within 30 days, notifies you in writing of any errors or omissions; you then have 21 days to respond. AHCA approves or denies within 60 days of a complete application. Fees are non-refundable, so submit a complete package.

Choosing your name and service area

Your business name must not include home health, certified nursing assistant, home health aide, personal care, patient care, health care, assisted living, nurses, nursing, medical, health professional, or any word implying services beyond the homemaker and companion scope. If you operate under any name other than your legal entity name, register that fictitious name with SunBiz first. Your service area is the county or counties within the health service planning district where your address of record is located; you can add counties within your district through the online system at no charge, but a separate office in a different area needs its own registration.

You can operate from home

Florida allows a homemaker and companion provider to operate from a home office, as long as it is within Florida and in the area you intend to serve. Note that AHCA may inspect on an unannounced basis during a weekday if it receives a complaint — so keep your records organized and your operating hours accurate (surveyors visit during the business hours you submit).

Registration steps

1. Form your legal entity with SunBiz and obtain your EIN; register a fictitious name if needed.
2. Confirm your business name avoids all restricted words.

3. Complete AHCA Form 3110-1003 in the online licensure system, selecting your service-area counties.
4. Complete Level 2 background screening (owners, administrator) and assemble the background-screening affidavit(s).
5. Complete HIV/AIDS training and prepare the HIV/AIDS Training Affidavit (Form 3110-1011).
6. Attach all required documents (per Chapters 408 Part II and 400 Part III, and Rules 59A-35 and 59A-8.025) and pay the fee.
7. Respond to any omissions letter within 21 days so the application is not withdrawn.
8. Receive your registration; verify your listing on Florida Health Finder and keep the certificate available for inspection.

What goes in a complete application

AHCA will not process an application that is missing fees or documents, so assemble a complete package. Typical components include your entity and fictitious-name proof from SunBiz, your EIN, controlling-interest information for owners and officers with a 5% or greater interest (defined in s. 408.803(7)), Level 2 background-screening results or the background-screening compliance affidavit, the HIV/AIDS Training Affidavit, your selected service-area counties, your operating hours (surveyors inspect during those hours), and the fee. Do not put Social Security numbers on the main form — they go on the separate Health Care Licensing Application Addendum (AHCA Form 3110-1024). A clean, complete first submission is the difference between a smooth 60-day approval and weeks of omissions letters.

Part 3 — Background Screening & Training

Two things AHCA cares about most for homemaker and companion providers are screening and training. Get these right and you are most of the way to a clean registration and survey.

Level 2 background screening (the big one)

Every owner, the administrator, and every homemaker and companion must complete Level 2 background screening — fingerprint-based, including a Florida Department of Law Enforcement (FDLE) state check and an FBI national check — before hiring and before entering any client's home, under Sections 400.512 and 408.809 and Chapter 435, Florida Statutes. Screening runs through the AHCA Care Provider Background Screening Clearinghouse: you get your AHCA file number, schedule electronic fingerprinting (Livescan) with an approved provider using that number, and enroll each person in the Clearinghouse for ongoing eligibility monitoring. A person with a disqualifying offense may not serve clients unless AHCA grants an exemption under s. 435.07. Screening costs are additional to the registration fee and vary by Livescan provider — verify current pricing.

The administrator's annual affidavit

Under s. 400.512, the administrator signs an affidavit each year, under penalty of perjury, that all personnel who enter clients' homes have been screened. Treat this as the serious legal attestation it is — never sign it until every active staff member's screening is verified and current in the Clearinghouse.

Required training

- **HIV/AIDS in-service (annual):** Staff complete HIV/AIDS training, and you submit the HIV/AIDS Training Affidavit (AHCA Form 3110-1011) at initial registration and each renewal.
- **Alzheimer's / ADRD training:** Any homemaker or companion who directly serves a client with Alzheimer's or a related dementia must complete ADRD training under s. 400.6045. Surveyors cite non-compliance often (citations CZ875/ZZ875), so track completion for every eligible staff member.
- **Emergency management training (annual):** Staff train yearly on evacuation, emergency communication, and disaster readiness — essential given Florida's hurricane exposure and frail, home-bound clients.

Recruiting and keeping good companions

Your business is only as good as the companions who show up. Recruit for reliability and warmth first — skills like meal prep and housekeeping are teachable, but dependability and kindness are not. Build a simple, repeatable hiring workflow: application, interview, Level 2 screening through the Clearinghouse, employment-history verification (the rule expects at least two documented attempts), orientation on the non-medical scope, and required training before the first shift. Retention is cheaper than recruiting: predictable schedules, respect, prompt pay, appreciation, and steady communication keep good staff. Because your workers go into homes alone, give them clear check-in expectations, permission to leave any unsafe situation, and a fast way to reach you — staff who feel supported stay, and stable staffing is exactly what referral partners and clients notice.

Part 4 — Setting Up to Operate

Beyond the registration itself, a few operational foundations make your business run smoothly and keep you survey-ready:

- **Policies & forms:** A Florida-specific Policy & Procedure Manual is one of the most scrutinized parts of readiness. Your manual, caregiver handbook, and forms (this package) show AHCA you understand and follow the rules — scope, screening, training, client rights, abuse reporting, emergency management, and documentation.
- **Records:** Keep organized personnel files (screening, training, affidavits, employment-history verification) and client files (service agreement, service plan, visit notes). Retain client/service records at least three years after the last entry.
- **Insurance:** Florida does not require insurance for the registration, but general liability (and bonding) is strongly recommended and reassures referral partners and clients. Carry workers' compensation where required (generally at four or more employees, non-construction).
- **Supervision:** Plan how you'll supervise staff — and document a supervision plan for any county outside your primary service area.

Your office, records, and privacy

You can operate from a Florida home office, but treat it like a real business location. Keep client and personnel records secure and organized — locked and access-limited for paper, password-protected for digital — because they contain sensitive personal information protected by HIPAA and because AHCA may inspect on an unannounced weekday visit if a complaint is received. Maintain separate, clearly labeled files: personnel (screening, training, affidavits, employment-history verification, acknowledgments) and client (intake, service agreement, individualized service plan, visit notes, correspondence). Post your operating hours accurately, since surveyors visit during them, and be ready to produce any record quickly.

Scheduling, coverage, and communication

Reliability is your product, so scheduling is a core competency, not an afterthought. Build a simple system to match clients with consistent companions, cover call-outs quickly, and confirm visits. Clients and families notice when the same trusted person arrives on time every week — and they notice, unforgivingly, when no one shows. Keep an accessible on-call plan, communicate schedule changes early, and document every visit so you can prove service, support billing, and catch problems before they grow.

Part 5 — Finding Clients & Referrals

Referrals are the lifblood of a home-care business. Elderly and disabled adults — and the people who help them make decisions — are concentrated around predictable places: hospital discharge planners, skilled-nursing and rehab facilities, hospices, assisted-living and senior communities, funeral homes, elder-law attorneys, and geriatric care managers and senior-placement agents. The companion Referral System (Florida Edition) gives you verified named partners with addresses and phone numbers across these categories for the major Florida metros, plus

how to approach each type. Start those relationships before you're licensed, so referrals are ready to flow the day you open.

Two rules keep referral-building lawful and durable. First, market truthfully — always describe your services as non-medical companion and homemaker services and never imply personal care or nursing. Second, earn referrals through quality and reliability, not payments: Florida's patient-brokering law (s. 817.505) and federal anti-kickback rules prohibit paying or receiving anything of value for referrals. Show up, communicate, and deliver — that is what makes a discharge planner or care manager send you the next client.

How to approach each referral source

- **Hospitals & rehab facilities** — ask for case management or discharge planning; be the reliable agency that accepts a referral quickly and reports back. Reducing readmissions is their goal, and dependable non-medical support at home helps.
- **Assisted living & senior communities** — connect with the director or community-relations lead; companions can supplement community staff with one-on-one time, outings, and family peace of mind.
- **Geriatric care managers & senior-placement agents** — they place clients into services daily and value a provider who communicates and delivers; earn a spot on their short list.
- **Elder-law attorneys & funeral homes** — they meet families at decision points and are trusted to recommend providers; a professional, honest relationship pays off over years.
- **Area Agencies on Aging / ADRCs** — the front door for seniors seeking help; understand their programs so you can serve referred clients and, where eligible, Medicaid-funded ones.

Building your own visibility

Referral relationships are your strongest channel, but families also search on their own — so be findable and credible. Set up a simple, professional website that clearly describes your non-medical companion and homemaker services (and never implies personal care or nursing), a Google Business Profile so you appear in local searches and maps, and a way for happy clients and families to leave reviews. Reviews and word of mouth carry enormous weight in senior care, where trust is everything. Show up in the community too: senior centers, faith communities, caregiver support groups, and local senior expos put you in front of exactly the families who need you. Keep all of it truthful and within scope — your reputation for honesty is part of what earns the next referral.

Part 6 — Getting Paid

Most homemaker and companion providers start primarily private pay — you set your hourly rates and bill clients or families directly, which is the simplest and fastest cash flow. As you grow, several other funding paths can be added:

Funding source	What it covers	How to access / notes
Private pay	Companion & homemaker hours, billed directly to the client or family	Your service agreement and rate schedule; no EVV; primary source for most new providers
Medicaid — SMMC Long-Term Care	Homemaker/companion for Medicaid-eligible seniors & adults with disabilities via managed-care plans	Enroll as a Medicaid provider and contract with LTC plans; EVV applies; verify current rates with the plan
Medicaid — APD iBudget waiver	Companion services for people with developmental disabilities	APD provider enrollment; note: companion-only providers contracted with APD for waiver clients are exempt from HCS registration
Veterans' programs	Some non-medical in-home support for eligible veterans	VA / Aid & Attendance and VA-directed care; verify eligibility and authorization

Funding source	What it covers	How to access / notes
Long-term-care insurance	Non-medical home care where the policy covers it	Bill per the client's policy; requirements vary by carrier

Electronic Visit Verification (EVV) — only if you bill Medicaid

If you bill Medicaid, federal law requires Electronic Visit Verification: an electronic record of each in-home visit capturing the service, client, provider, date, location, and start/end times. Florida uses an Open Vendor Model — for fee-for-service Medicaid, AHCA's system is HHAeXchange (go-live October 1, 2024); managed-care plans use Netsmart/Tellus or HHAeXchange depending on the plan. You may use a compliant third-party system that integrates with the required aggregator. Monitor your EVV compliance percentage, because unverified visits can be denied. If you serve only private-pay clients, EVV does not apply — but you still document every visit.

What it costs to get started (verify current figures)

Item	Approx. amount	Notes
AHCA HCS registration	\$50.75 (2-year)	Verify current; late fee recently \$25.38 if under 60 days before expiration
Level 2 screening (per person)	Varies by Livescan provider	Separate from registration; fingerprint + FDLE/FBI; verify current pricing
Business formation (SunBiz LLC)	State filing fee	Verify current at dos.fl.gov / SunBiz
Fictitious name (if used)	State filing fee	Only if operating under a name other than your legal entity name
Insurance (optional but advised)	Varies	General liability and/or bond; workers' comp where required

Dollar figures reflect amounts published by AHCA and the state at the time of writing; confirm current fees before you rely on them.

Setting your private-pay rates

Since most new providers start private-pay, your rates matter. Research what comparable non-medical companion and homemaker providers charge per hour in your county, and set an hourly rate that covers caregiver wages, payroll taxes, workers' comp and insurance, supervision and office time, and a reasonable margin. Many agencies set a minimum visit length (for example, a few hours) to make scheduling and travel worthwhile, and charge for mileage on transportation and errands. Put your rates, minimums, billing cycle, and payment terms in your service agreement up front so clients and families know exactly what to expect — clarity prevents disputes and protects you from any appearance of exploitation.

Getting paid on time

Cash flow makes or breaks a young agency. Invoice on a predictable schedule, offer easy payment methods, and follow up promptly on late accounts. For any client funds you handle for errands, keep receipts, return exact change, and document every transaction under your client-funds policy. If you add Medicaid, remember that payment depends on authorization, correct billing, and EVV compliance — unverified or unauthorized visits do not get paid — so build those habits before you take your first Medicaid client.

Veterans' benefits and long-term-care insurance

Two funding sources are worth understanding as you grow. Many veterans and surviving spouses qualify for VA support — including the Aid & Attendance enhancement to a VA pension and VA-directed home and community-based care — that can pay for non-medical in-home help; eligibility and authorization vary, so verify with the VA and the family before promising coverage. Separately, many older adults hold long-term-care insurance policies

that reimburse non-medical home care once a benefit trigger is met; these pay per the client's specific policy, often on a reimbursement basis with documentation requirements, so read the policy and keep clean visit records. Neither source is a fast on-ramp, but both can add stable, well-paying clients to a private-pay base.

Part 7 — Your First 90 Days Operating

Getting registered is the start; the first 90 days are where you build a business that lasts. A simple plan:

1. Days 1–15: Finalize your Policy & Procedure Manual, caregiver handbook, and forms; set your rate schedule and service agreement; confirm your Clearinghouse enrollment and training records are complete.
2. Days 1–30: Launch referral outreach using the Referral System — introduce yourself to discharge planners, senior communities, care managers, and elder-law attorneys in your metro.
3. Days 15–45: Recruit and screen your first homemakers and companions; complete HIV/AIDS and (as needed) ADRD and emergency-management training; build your scheduling and documentation routine.
4. Days 30–60: Onboard your first clients; complete intake, service agreements, and individualized service plans; begin supervisory contacts and visit documentation.
5. Days 45–75: If pursuing Medicaid, start provider enrollment and EVV setup; otherwise refine private-pay billing and collections.
6. Days 60–90: Run your first quality review — check documentation, scope compliance, and client satisfaction; fix gaps; ask happy clients and partners for referrals and reviews.
7. Ongoing: Calendar your renewal (submit 60+ days early), track training due dates, and prepare for a possible unannounced complaint survey by keeping records inspection-ready.

Part 8 — Common Mistakes to Avoid

- **Drifting into personal care.** The single biggest risk. A client asks for help into the shower and a kind caregiver obliges — and now you're doing unlicensed home health work. Train staff hard on the boundary and refer out when needs change.
- **Letting someone work before screening clears.** No one enters a client's home until Level 2 screening is confirmed eligible in the Clearinghouse. Never assume a pending result will clear.
- **Signing the annual affidavit without reconciling screening.** It's sworn under penalty of perjury — verify every active staff member first.
- **A non-compliant business name.** Using 'health,' 'nursing,' 'personal care,' etc. will hold up your registration; check the restricted-words list before you file or brand.
- **Missing training deadlines.** Lapsed HIV/AIDS, ADRD, or emergency-management training is a common citation. Track due dates in a training matrix.
- **Renewing late.** Submit at least 60 days before expiration; a lapse forces you to stop operating and re-apply as an initial applicant.
- **Paying for referrals.** Florida's patient-brokering law and federal anti-kickback rules prohibit it — earn referrals with quality instead.
- **Sloppy documentation.** Thin visit notes and disorganized files sink you in a complaint survey and in billing disputes. Document every visit and keep files three years.

Part 9 — Staying Compliant Year-Round

Registration is not a one-time event; it is a standard you maintain. Build a simple compliance rhythm so nothing lapses and a surprise survey is a non-event:

- **Keep a compliance calendar.** Track your registration expiration (renew at least 60 days early), every staff member's screening eligibility, and all training due dates (HIV/AIDS, ADRD, emergency management).

- **Reconcile before the annual affidavit.** Before the administrator signs the s. 400.512 affidavit, confirm every active staff member is screened and eligible in the Clearinghouse.
- **Run a quarterly quality review.** Check documentation completeness, scope compliance (no drift into personal care), complaints, incidents, and client satisfaction; fix gaps and retrain.
- **Be survey-ready every day.** AHCA may inspect unannounced during business hours if a complaint is received. Keep records producible and do a periodic mock inspection.
- **Report what the law requires.** Train staff to call the Florida Abuse Hotline immediately on any suspicion of abuse, neglect, or exploitation, and report changes (address, administrator, ownership) to AHCA on time.
- **Update your policies when rules change.** Review your manual at least annually and whenever a statute or rule changes, and record the review.

If AHCA identifies a deficiency during a survey, respond professionally: prepare and implement a plan of correction within the required timeframe, document the fix, and verify it holds. A calm, well-documented response to a complaint survey protects your registration and your reputation — and reassures the referral partners who send you clients.

Worked Example 1 — Miami-Dade (South Florida)

Suppose you're launching in Miami-Dade, one of Florida's largest and most diverse senior markets. You form your LLC with SunBiz, choose a compliant name (for example, 'Sunshine Companion Care of Miami LLC' — no restricted words), and set your service area to Miami-Dade within your health service planning district. You Level 2 screen yourself and your first two companions through the Clearinghouse, complete HIV/AIDS training, and file AHCA Form 3110-1003 online with the \$50.75 fee. While AHCA reviews (up to 60 days), you build referral relationships — hospital discharge planners, assisted-living communities, and care managers from your Referral System — and you connect with the Alliance for Aging, Inc., the Area Agency on Aging and Aging & Disability Resource Center for Miami-Dade and Monroe, whose Elder Helpline (305-670-4357; main office 305-670-6500; toll-free 1-800-96-ELDER) is a hub for seniors seeking exactly the non-medical help you provide. You serve a growing multilingual client base, staying strictly companion-and-homemaker, and refer any hands-on personal-care need to a licensed home health partner.

Worked Example 2 — Tampa Bay

Now suppose you're opening in the Tampa Bay area. Same foundation: SunBiz LLC, compliant name, Tampa/Hillsborough service area, Clearinghouse screening, HIV/AIDS training, and the online AHCA application with the \$50.75 fee. You focus on post-hospital support and senior companionship, and you build referral relationships with Tampa hospitals, rehab facilities, and senior communities from your Referral System. You connect with Senior Connection Center, Inc., the Area Agency on Aging and ADRC for West Central Florida (813-740-3888; 1-800-96-ELDER), and with Hillsborough County Aging Services (813-272-5250), both of which field constant requests for reliable in-home, non-medical help. As you add Medicaid Long-Term Care clients through managed-care plans, you set up EVV with the plan's aggregator and keep private-pay as your steady base.

Frequently Asked Questions

Q: Do I need a license, or just a registration?

A: A registration. Homemaker and companion providers register with AHCA under s. 400.509 — it is not a health-care license. But that registration strictly limits you to non-medical services; hands-on personal care requires a separate Home Health Agency license.

Q: Can I provide any personal care at all — even a little?

A: No. AHCA is explicit that this registration does not allow hands-on personal care: no bathing, dressing, feeding, toileting, transferring, or medication assistance. Even occasional help crosses the line — refer those needs to a licensed home health agency or nurse registry.

Q: Can I run the business from home?

A: Yes, as long as your home office is in Florida and in the area you serve. Be aware AHCA may inspect on an unannounced weekday if a complaint is received, and surveyors visit during your posted operating hours.

Q: How much does it cost and how long does it take?

A: The registration fee is \$50.75 for two years (verify current), plus separate background-screening costs and business-formation fees. AHCA decides within 60 days of a complete application, after a 30-day window to flag omissions that you have 21 days to answer.

Q: Do I need insurance?

A: Not to register — Florida does not require it for the registration. But liability insurance (and bonding) is strongly recommended, and workers' compensation is generally required once you have four or more employees (non-construction).

Q: Do I have to use EVV?

A: Only if you bill Medicaid. EVV is required for Medicaid in-home visits under an Open Vendor Model (HHAeXchange for fee-for-service; plan-specific for managed care). Private-pay-only providers are not subject to EVV but still document every visit.

Q: I already have a Home Health Agency or Nurse Registry license — do I also need this?

A: No. A licensed Home Health Agency or Nurse Registry may provide homemaker and companion services under its existing license and is exempt from this separate registration.

Statewide Contacts

For...	Who to contact
Registration, rules & inspections	AHCA Long-Term Care Services Unit / Bureau of Health Facility Regulation — (850) 412-4303; LTCStaff@ahca.myflorida.com; 2727 Mahan Drive, MS #33, Tallahassee, FL 32308
Online application (Form 3110-1003)	AHCA online licensure system (ahca.myflorida.com → Health Care Policy & Oversight → Online Licensure)
Public provider directory	Florida Health Finder — quality.healthfinder.fl.gov
Consumer complaints about providers	AHCA Consumer Complaint — 1-888-419-3456
Suspected abuse, neglect, or exploitation	Florida Abuse Hotline — 1-800-96-ABUSE (1-800-962-2873)
Business formation & fictitious names	Florida Division of Corporations (SunBiz) — (850) 245-6000; dos.fl.gov
Medicaid EVV support	AHCA EVV Customer Support — 1-833-AHCA-EVV (1-833-242-2388)
Medicaid provider enrollment / EDI	Florida Medicaid — 1-800-289-7799
Elder resources & referrals (statewide)	Florida Dept. of Elder Affairs Elder Helpline — 1-800-96-ELDER (1-800-963-5337)
Miami-Dade & Monroe aging network	Alliance for Aging, Inc. — 305-670-6500; Elder Helpline 305-670-4357
Tampa Bay / West Central FL aging network	Senior Connection Center, Inc. — (813) 740-3888; Hillsborough County Aging Services (813) 272-5250
Orlando aging network	Senior Resource Alliance — (407) 514-0019
Provider association	Home Care Association of Florida (HCAF) — education, advocacy, and networking

Glossary

- **AHCA** — Florida Agency for Health Care Administration, which registers and oversees homemaker and companion providers.
- **Homemaker & Companion Services (HCS)** — the non-medical registration under s. 400.509 for companionship, supervision, and homemaking; no hands-on personal care.
- **Level 2 screening** — fingerprint-based FDLE + FBI background screening under Chapter 435, run through the AHCA Clearinghouse.
- **Clearinghouse** — the AHCA Care Provider Background Screening Clearinghouse that stores results and monitors eligibility.
- **ADRD training** — Alzheimer's Disease and Related Dementia training under s. 400.6045 for staff serving clients with dementia.
- **SMMC-LTC** — Statewide Medicaid Managed Care Long-Term Care, through which plans pay for homemaker/companion services.
- **APD iBudget waiver** — Agency for Persons with Disabilities waiver funding companion services for people with developmental disabilities.
- **EVV** — Electronic Visit Verification, required for Medicaid in-home visits; Florida uses an Open Vendor Model (HHAeXchange / Netsmart).
- **Home Health Agency (HHA)** — the Chapter 400, Part III license required to provide hands-on personal care or skilled services — your future growth path.

Core Rules & References

- Section 400.509, Florida Statutes — registration of homemaker and companion services providers.
- Rule 59A-8.025, Florida Administrative Code — registration requirements for homemaker and companion services providers.
- Chapter 408, Part II, Florida Statutes — health care licensing procedures (application, fees, renewal, change of ownership, inspection, enforcement).
- Section 400.512 & Chapter 435, Florida Statutes — Level 2 background screening and the administrator's annual affidavit.
- Section 400.6045, Florida Statutes — Alzheimer's / ADRD training.
- Section 817.505, Florida Statutes — patient brokering (anti-kickback) prohibition.
- AHCA Form 3110-1003 (application) and Form 3110-1011 (HIV/AIDS Training Affidavit).

Part 10 — Where to Go From Here

Starting as a homemaker and companion provider is the smart on-ramp into Florida's home-care economy: it is affordable, achievable, and it generates cash flow and referral relationships quickly. But it is also a foundation you can build on. Once you are established and your systems are humming, three growth paths open up.

- **Add Medicaid clients.** Enroll as a Medicaid provider and contract with Statewide Medicaid Managed Care Long-Term Care plans (or serve APD waiver clients) to add a steady, funded client base alongside your private-pay work — building the EVV and documentation discipline those programs require.
- **Deepen a niche.** Become the go-to provider for dementia companionship, post-hospital support, or a particular community. Depth earns referrals and lets you command better rates than a generalist.
- **Grow into licensed care.** When you are ready to offer hands-on personal care or skilled services, pursue a Home Health Agency license under Chapter 400, Part III. It is a larger undertaking — application fee, surety bond, Director of Nursing, and a pre-licensure survey — but your reputation, referral network, and

operational experience from this business make it far more reachable. That is the path we will build with you when you are ready.

Whatever path you choose, the fundamentals are the same ones this package is built around: stay strictly within your scope, screen and train every caregiver, document everything, treat clients and staff with respect, and keep your compliance airtight. Do those things well and the growth takes care of itself.

This guide is a general educational resource for prospective Florida homemaker and companion providers. It is not legal, tax, or billing advice, and requirements, fees, and contacts change. Verify every step, figure, and phone number with AHCA, the Florida Division of Corporations, and the relevant payer before you rely on it. Provisions cited reflect Florida law and AHCA guidance at the time of writing.